BASELINE → TREATMENT → FOLLOW-UP

Checks throughout treatment

Relative stages; no dates need to be entered. Ask who arranges and reviews each check.

Check	Before starting	Cycle 1 · Day 1	Cycle 1 · Day 22	Later cycles · Day 1	Later cycles · Day 22	Completion / follow-up
Treatment	Confirm indication and plan	All four medicines	Nivolumab + final paclitaxel/ carboplatin	Nivolumab + ipilimumab	Nivolumab alone	Agree stop plan and continuing care
Blood count, kidney and liver	Baseline results	Review before treatment	Review before treatment	Review before every cycle	Review before treatment	Follow unresolved toxicity
Hormones and immune symptoms	Glucose, thyroid function tests (TFT); hepatitis screen; cortisol if indicated	Glucose/TFT before cycle and symptom review	Review symptoms; extra tests if needed	Glucose/TFT before cycle and symptom review	Review symptoms; extra tests if needed	Vigilance at least five months after last dose
Support medicines	Paclitaxel premedication includes corticosteroid and antihistamine treatment, with H2-blocker decisions made by the team. The source allows consultant-directed changes; ask for a written plan for each of the two chemotherapy visits.	Chemotherapy premedication/antiemetics	Repeat prescribed chemotherapy support	Low-risk support; no chemotherapy prophylaxis	Minimal-risk support; no routine prophylaxis	Confirm medicines and steroid taper if used
Response and preferences	Discuss goals and alternatives	Ask about symptoms and priorities	Review treatment experience	Consultant-directed scans and benefit review	Report new concerns between visits	Agree responsible team and symptom support

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00712 v3a · official regimen

THE WHOLE TREATMENT JOURNEY

Nivolumab, ipilimumab, paclitaxel and carboplatin: treatment and checks

First-line metastatic squamous non-small-cell lung cancer (NSCLC) in adults without sensitising EGFR mutation or ALK translocation.

Cycle 1 lasts 42 days: nivolumab, ipilimumab, paclitaxel and carboplatin on Day 1; nivolumab, paclitaxel and carboplatin on Day 22. These are the only two planned chemotherapy administrations. From Cycle 2, nivolumab is given on Days 1 and 22 and ipilimumab on Day 1. Continue while appropriate until progression, unacceptable toxicity or up to 24 months without progression. Your team reviews the plan and may delay or stop treatment.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Cycle 1 · Day 1	Cycle 1 · Day 22	Later cycles · Day 1	Later cycles · Day 22	Completion / follow-up
Treatment NCCP 00712	Confirm indication and plan	All four medicines	Nivolumab + final paclitaxel/ carboplatin	Nivolumab + ipilimumab	Nivolumab alone	Agree stop plan and continuing care
Blood count, kidney and liver NCCP 00712	Baseline results	Review before treatment	Review before treatment	Review before every cycle	Review before treatment	Follow unresolved toxicity
Hormones and immune symptoms NCCP 00712	Glucose, TFT; hepatitis screen; cortisol if indicated	Glucose/TFT before cycle and symptom review	Review symptoms; extra tests if needed	Glucose/TFT before cycle and symptom review	Review symptoms; extra tests if needed	Vigilance at least five months after last dose
Support medicines NCCP 00712	Paclitaxel premedication includes corticosteroid and antihistamine treatment, with H2-blocker decisions made by the team. The source allows consultant-directed changes; ask for a written plan for each of the two chemotherapy visits.	Chemotherapy premedication/antiemetics	Repeat prescribed chemotherapy support	Low-risk support; no chemotherapy prophylaxis	Minimal-risk support; no routine prophylaxis	Confirm medicines and steroid taper if used
Response and preferences NCCP 00712	Discuss goals and alternatives	Ask about symptoms and priorities	Review treatment experience	Consultant-directed scans and benefit review	Report new concerns between visits	Agree responsible team and symptom support

What each check is for

1 Baseline

Before first infusion

Confirm exact eligibility

Ask / **act:** Review results, consent, medicines and contacts.

Sources: NCCP 00712

2 Day 22

Three weeks after Day 1

Different active medicines

Ask / **act:** No ipilimumab on Day 22; confirm support plan.

Sources: NCCP 00712

3 Continuation

Later 42-day cycles

Review benefit and toxicity

Ask / **act:** Apply joint immune hold rules and review duration.

Sources: NCCP 00712

4 End of treatment

At progression/toxicity or up to 24 months

Late immune effects can occur

Ask / **act:** Document follow-up and urgent contacts.

Sources: NCCP 00712

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-29 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.27